

D348

Psychiatric Mental Health Nurse Practitioner Clinical Internship I

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 45-year-old accountant who recently bought a new car reports four weeks of low mood, fatigue, weight gain, constipation, and always feeling cold. She has no psychiatric history. Her preceptor asks the student for next steps. What should come first?

- A. Check thyroid function (TSH) and other basic labs before confirming a primary depressive disorder.
- B. Diagnose major depressive disorder and start an SSRI today.
- C. Tell her the symptoms are normal stress from a big purchase.
- D. Order brain imaging for every new depression.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Which presentation most strongly suggests a medical or neurological cause rather than a primary psychotic disorder?

- A. Any psychosis, since all psychosis is medical.
- B. Auditory hallucinations beginning at age 20 with clear consciousness.
- C. Paranoid beliefs in a patient with a family history of schizophrenia.
- D. First-ever psychosis at age 62 with visual hallucinations and fluctuating confusion.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A 30-year-old graduate student with passive suicidal thoughts and no current plan is being discharged from urgent care. She rides her bike to campus and has a cat. The preceptor asks the student to address safety. Which action is evidence-based?

- A. Suggest caring for her cat as her main coping strategy.
- B. Tell her to call if she ever develops a plan, with no other steps.
- C. Develop a written safety plan with her: warning signs, coping steps, contacts, crisis lines, and lethal means safety.
- D. Have her sign a no-suicide contract.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A new patient says he receives methadone at a federally assisted opioid treatment program. The preceptor wants his dosing records. The clinic is located in the same building as the program and shares a parking lot. What is required?

- A. Nothing; HIPAA allows sharing for treatment.
- B. A court order in every case.
- C. The patient's written consent that meets 42 CFR Part 2 before the program releases records.
- D. Nothing, since the clinics share a building.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

Before starting olanzapine, a patient weighs 82 kg and is 1.75 m tall. His waist is 94 cm and blood pressure is 124/80. What is his baseline BMI?

- A. 30.7
- B. 0.037
- C. 26.8
- D. 46.9

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A patient is 1.60 m tall. What is the highest weight at which her BMI stays at 25?

- A. 40 kg
- B. 64 kg
- C. 9.8 kg
- D. 25 kg

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A student NP has completed an assessment and wants to start a patient on sertraline. The preceptor is with another patient for 20 minutes, and the student has a 4.0 GPA. What should the student do?

- A. Prescribe because strong grades show competence.
- B. Tell the patient no plan can be made during clinicals.
- C. Send the prescription now to save time.
- D. Present the plan to the preceptor for review and have the preceptor approve and sign any prescription.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A 28-year-old chef had 5 days of euphoria, little sleep, and reckless spending that ended in psychiatric hospitalization. He has never had a major depressive episode. He enjoys hiking on weekends. What is the diagnosis?

- A. Bipolar I disorder
- B. Bipolar II, because the episode lasted under 7 days
- C. No bipolar diagnosis without a depressive episode
- D. Cyclothymic disorder

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A patient has had two major depressive episodes and one 4-day period of elevated mood with less need for sleep that did not impair work or require hospitalization. Which diagnosis fits?

- A. Bipolar I disorder
- B. Bipolar II disorder
- C. Cyclothymic disorder
- D. Major depressive disorder

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A patient who drives for a delivery company says, “I know smoking weed every night isn’t great, but it’s the only thing that helps me sleep.” He has two brothers. Which response reflects motivational interviewing?

- A.** “Part of you is worried about it, and part of you relies on it for sleep.”
- B.** “You need to stop right away or you’ll ruin your life.”
- C.** “Do your brothers use too?”
- D.** “Just take a sleeping pill instead.”

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A student writes a SOAP note for a patient who wore a bright orange jacket. She documents: patient appears disheveled, speech is rapid and pressured, affect is expansive, and PHQ-9 score is 3. The patient came by bus. Where do these findings belong?

- A.** Subjective
- B.** Assessment
- C.** Plan
- D.** Objective

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

Which entry belongs in the Assessment section of a SOAP note?

- A.** Start lithium; recheck level in 5 days.
- B.** Bipolar I disorder, current episode manic, moderate, based on pressured speech, decreased sleep, and grandiosity.
- C.** BP 128/82, pulse 96.
- D.** Patient reports sleeping 3 hours a night.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A 45-year-old accountant ~~who recently bought a new car~~ reports four weeks of low mood, fatigue, weight gain, constipation, and always feeling cold. She has no psychiatric history. Her preceptor asks the student for next steps. What should come first?

Target word: rule out medical cause **Decides:** Physical symptoms suggest hypothyroidism.

A	Correct	Hypothyroidism can mimic depression.
B	Dropped element	Skips the medical rule-out.
C	Camouflage	The car purchase does not explain the physical signs.
D	Absolute language	Workup should be guided by findings.

2. Answer D Concept 1 • transfer

Which presentation most strongly suggests a medical or neurological cause rather than a primary psychotic disorder?

Target word: reverse: red flags for secondary cause **Decides:** Late onset, visual hallucinations, fluctuating confusion.

A	Absolute language	Primary psychotic disorders are common.
B	Inverted operation	Typical of primary psychosis.
C	Adjacent category	Family history raises primary risk.
D	Correct	Late onset, visual hallucinations, and confusion point to a medical cause.

3. Answer C Concept 2

A 30-year-old graduate student with passive suicidal thoughts and no current plan is being discharged from urgent care. ~~She rides her bike to campus and has a cat.~~ The preceptor asks the student to address safety. Which action is evidence-based?

Target word: safety intervention **Decides:** Safety planning, not contracts.

A	Camouflage	A pet may help, but it is not a full plan.
B	Dropped element	Leaves out the components of a safety plan.
C	Correct	Collaborative safety planning is evidence-based.
D	Confusion set	Contracts are not effective and can give false reassurance.

4. Answer C Concept 3

A new patient says he receives methadone at a federally assisted opioid treatment program. The preceptor wants his dosing records. ~~The clinic is located in the same building as the program and shares a parking lot.~~ What is required?

Target word: consent for SUD records **Decides:** Part 2 protects program records.

A	Adjacent category	Part 2 is stricter than HIPAA for SUD program records.
B	Absolute language	Patient consent is the usual route.
C	Correct	Part 2 requires patient consent for disclosure.
D	Camouflage	Location does not waive consent.

5. Answer C Concept 4

Before starting olanzapine, a patient weighs 82 kg and is 1.75 m tall. ~~His waist is 94 cm and blood pressure is 124/80.~~ What is his baseline BMI?

Target word: BMI **Decides:** $82 / 1.75^2$.

A	Wrong stem number	Uses the 94 cm waist: $94 / 3.0625$.
B	Inverted operation	Divides height squared by weight.
C	Correct	$82 / 3.0625 = 26.8$.
D	Dropped element	$82 / 1.75$ does not square the height.

6. Answer B Concept 4 • transfer

A patient is 1.60 m tall. What is the highest weight at which her BMI stays at 25?

Target word: solve for weight **Decides:** $\text{Weight} = \text{BMI} \times \text{height}^2$.

A	Dropped element	25×1.60 forgets to square height.
B	Correct	$25 \times 2.56 = 64$ kg.
C	Inverted operation	$25 / 2.56$ divides instead of multiplies.
D	Wrong stem number	The BMI value itself, not a weight.

7. Answer D Concept 5

A student NP has completed an assessment and wants to start a patient on sertraline. The preceptor is with another patient for 20 minutes, ~~and the student has a 4.0 GPA.~~ What should the student do?

Target word: scope of practice **Decides:** Only the licensed preceptor can prescribe.

A	Camouflage	Grades do not grant prescribing authority.
B	Adjacent category	Students can propose plans for preceptor approval.
C	Inverted operation	Exceeds student scope.
D	Correct	Students work under the preceptor's license and oversight.

8. Answer A Concept 6

A 28-year-old chef had 5 days of euphoria, little sleep, and reckless spending that ended in psychiatric hospitalization. He has never had a major depressive episode. ~~He enjoys hiking on weekends.~~ What is the diagnosis?

Target word: bipolar I vs. II **Decides:** Hospitalization makes it mania regardless of duration.

A	Correct	A manic episode requiring hospitalization is enough; depression is not required.
B	Dropped element	Ignores that hospitalization removes the duration rule.
C	Confusion set	Depression is required for bipolar II, not bipolar I.
D	Adjacent category	Cyclothymia involves milder, chronic symptoms.

9. Answer B Concept 6 · transfer

A patient has had two major depressive episodes and one 4-day period of elevated mood with less need for sleep that did not impair work or require hospitalization. Which diagnosis fits?

Target word: reverse: identify bipolar II **Decides:** Hypomania plus MDE, no mania.

A	Inverted operation	Requires a full manic episode.
B	Correct	Hypomania (≥ 4 days, no marked impairment) plus MDE.
C	Confusion set	Cyclothymia excludes full MDEs.
D	Dropped element	Ignores the hypomanic episode.

10. Answer A Concept 7

A patient ~~who drives for a delivery company~~ says, "I know smoking weed every night isn't great, but it's the only thing that helps me sleep." ~~He has two brothers.~~ Which response reflects motivational interviewing?

Target word: MI response **Decides:** Reflect ambivalence.

A	Correct	A double-sided reflection evokes his own reasons.
B	Inverted operation	The righting reflex increases resistance.
C	Camouflage	Shifts focus away from his ambivalence.
D	Adjacent category	Advice-giving skips exploring motivation.

11. Answer D Concept 8

A student writes a SOAP note for a patient ~~who wore a bright orange jacket~~. She documents: patient appears disheveled, speech is rapid and pressured, affect is expansive, and PHQ-9 score is 3. ~~The patient came by bus.~~ Where do these findings belong?

Target word: SOAP section **Decides:** Clinician observations and scores.

A	Confusion set	Subjective is the patient's own report.
B	Adjacent category	Assessment is the diagnostic reasoning drawn from findings.
C	Inverted operation	Plan is what will be done next.
D	Correct	MSE observations and scale scores are objective.

12. Answer B Concept 8 · transfer

Which entry belongs in the Assessment section of a SOAP note?

Target word: reverse: find assessment **Decides:** Diagnostic reasoning.

A	Adjacent category	Treatment belongs in Plan.
B	Correct	Diagnosis with supporting reasoning.
C	Inverted operation	Measurements are objective.
D	Confusion set	Patient report is subjective.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owled.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	6	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	5	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Rule out medical and substance causes	Before settling on a primary psychiatric diagnosis, rule out medical, medication, and substance causes (e.g., thyroid, B12, infection, neurological disease), especially with atypical or late onset.	Diagnosing a primary mood or psychotic disorder without a medical workup.	New psychiatric symptoms with physical clues or onset after midlife.
2	Safety planning	A collaborative written safety plan (warning signs, coping steps, contacts, professional help, lethal means safety) is evidence-based; no-suicide contracts are not.	Relying on a signed no-harm contract.	Discharge or follow-up for a patient with suicidal thoughts.
3	Consent for substance use treatment records	Records from federally assisted substance use disorder programs are protected by 42 CFR Part 2 and generally require the patient's written consent to disclose, which is stricter than routine HIPAA treatment disclosures.	Assuming HIPAA treatment rules let any provider obtain SUD program records.	A request for records from an opioid treatment or addiction program.
4	BMI and metabolic monitoring	$BMI = \text{weight (kg)} / \text{height (m)}^2$; second-generation antipsychotics need baseline and follow-up weight/BMI, waist, blood pressure, glucose, and lipids.	Dividing by height without squaring it.	Weight in kg and height in meters for a patient starting an antipsychotic.
5	Student scope under preceptorship	A student NP assesses and proposes plans, but the licensed preceptor must review, approve, and sign orders and prescriptions.	Acting independently because the student feels confident or the preceptor is busy.	A student ready to send an order or prescription.
6	Bipolar I vs. bipolar II	Bipolar I requires at least one manic episode (≥ 7 days or any duration if hospitalized); bipolar II requires at least one hypomanic episode (≥ 4 days) and one major depressive episode, with no mania ever.	Calling a hospitalized mania hypomania, or requiring depression for bipolar I.	Duration, severity, and hospitalization of an elevated mood episode.
7	Motivational interviewing	MI uses open questions, affirmations, reflections, and summaries to evoke the patient's own reasons for change; arguing for change (the righting reflex) increases resistance.	Lecturing or warning to persuade the patient.	A patient expressing mixed feelings about change.

#	Concept	Rule	Trap	Tell
8	SOAP documentation	Subjective = what the patient reports; Objective = what the clinician observes and measures (MSE, vitals, labs, scale scores); Assessment = diagnostic reasoning; Plan = treatment and follow-up.	Putting MSE findings in Subjective.	Where a piece of information belongs in the note.

Course scope (public description): Through precepted clinical experiences, the learner will develop the competencies needed to provide comprehensive and holistic mental health care to individuals, families, and communities across the lifespan. In the precepted clinical setting, the learner will combine competencies developed in preparatory advanced practice coursework to deliver consumer- centered mental health care. Therefore, the learner will conduct advanced mental health assessments and utilize the competencies of advanced pathophysiology, psychopharmacology, psychotherapy, and health promotion for individuals and populations across the life span to build self-efficacy in individuals and groups, to determine correct diagnoses, and to establish mental health plans of care that include consumer and population preferences. All MSN Core Courses, NP Core courses, and PMHNP Specialty courses must be completed before taking this course.